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AI WILL BE A FORCE MULTIPLIER IN SHAPING REAL CLINICAL OUTCOMES AND TRANSFORMING HOW HEALTHCARE REACHES THE UNDERSERVED WHILE FORCING HARD QUESTIONS ON REGULATION

Artificial Intelligence has crossed a critical threshold in healthcare. We are no longer debating whether AI can work in clinical care—we are now grappling with how it should work, who it should serve, and under what guardrails it must operate. As we move into 2026, AI promises to reshape diagnostics, personalise treatment pathways and even influence how clinical trials and experiments are designed. Yet alongside this promise lie fundamental challenges around liability, regulation, trust and the evolving role of clinicians.

AI's earliest successes in healthcare were largely technical—recognising patterns in images, signals and large datasets faster than humans could. Today, that capability is translating into real clinical value. In radiology and pathology, AI systems are demonstrating their ability to flag subtle abnormalities, prioritise high-risk cases and reduce diagnostic delays, particularly in primary care and preventive health. AI is enabling risk stratification at scale—identifying individuals who may otherwise never enter the healthcare system until disease has progressed.

At Niramai, we have seen this firsthand through Thermalytix, an AI-based breast cancer screening solution that combines thermal imaging with machine learning to detect early physiological changes associated with malignancy. The technology itself is important, but the larger lesson is this: AI can extend clinical reach. It can bring screening and triage closer to where people live and work, rather than forcing healthcare to remain confined within hospital walls.

India has fewer than one doctor per 1,000 people, and specialist availability drops precipitously outside urban centres. Screening rates for common cancers remain among the lowest globally, and preventable conditions are often detected far too late. Against this backdrop, AI's most powerful contribution will not be novelty, but reach—extending clinical intelligence to where clinicians are scarce and healthcare infrastructure is limited.

Augmenting Care in Scarce Settings

In the coming decade, AI in Indian healthcare will function primarily as a force multiplier. In clinical settings, AI-driven tools can help prioritise high-risk patients, reduce diagnostic oversight and support faster decision-making—particularly in radiology, pathology and triage-heavy specialties. In non-clinical or semi-clinical environments, AI enables task-shifting, empowering nurses, technicians and community health workers to perform early screening and risk stratification that previously required specialist expertise. This distinction is critical. In India, AI is not competing with clinicians; it is compensating for their absence. The question is not whether AI will replace radiologists or pathologists, but whether AI can ensure that patients who most need a specialist actually reach one in time.

Breast cancer offers a clear illustration of both the problem and the opportunity. Despite being the most common cancer among Indian women, screening uptake remains alarmingly low—especially in rural and peri-urban regions. Traditional screening modalities are expensive, infrastructure-heavy and dependent on skilled operators. As a result, most diagnoses occur at advanced stages, where outcomes are poorer and treatment costs soar.

AI-powered screening tools, particularly those designed for low-resource settings, can dramatically alter this equation. By combining non-invasive imaging with machine learning–based risk assessment, such tools allow trained

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